

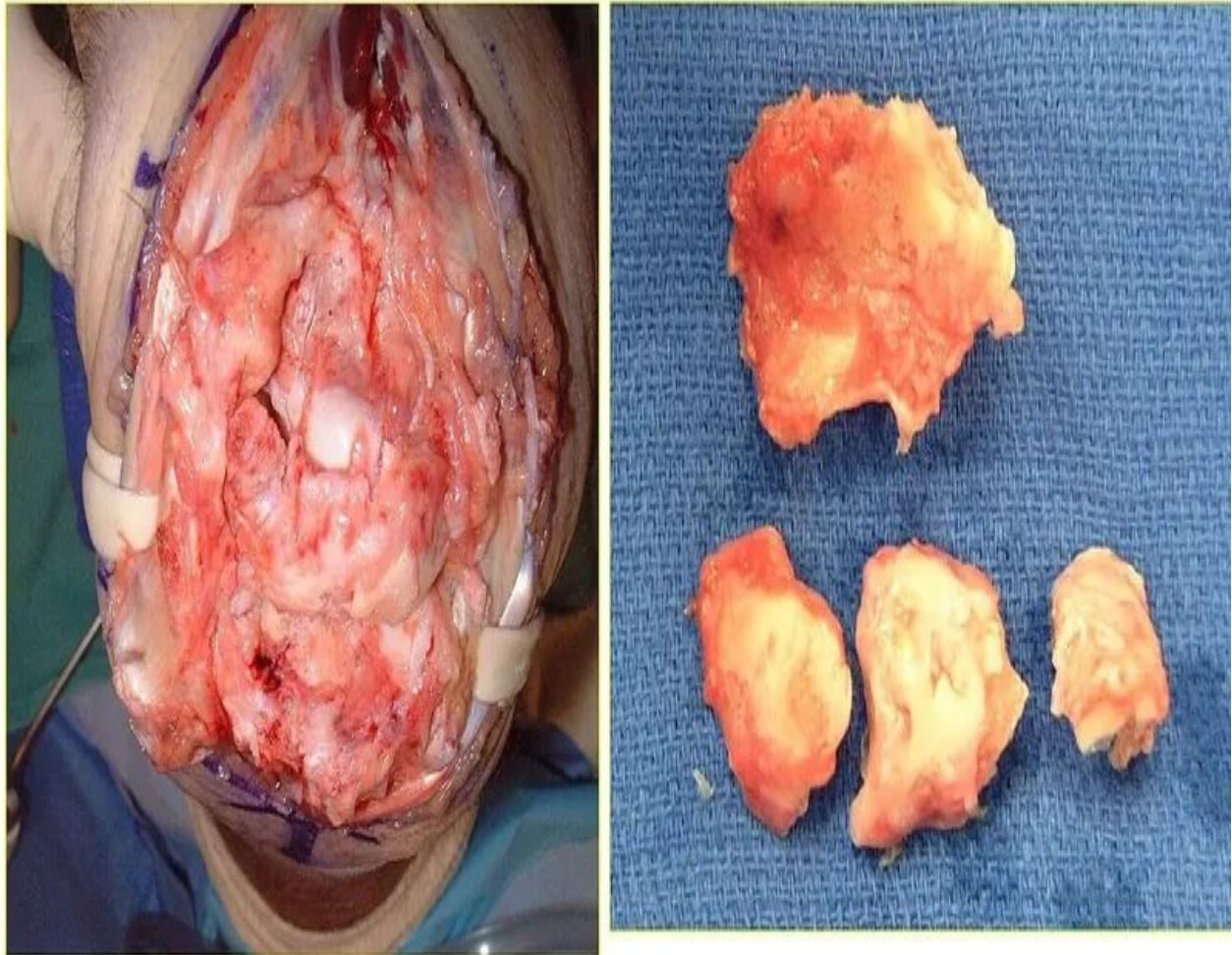
# Preventing Falls: A Guide for Frail and At-Risk Adults

Falls are a leading cause of injury in older adults, but most are preventable. Practical, evidence-based steps to reduce fall risk at home and beyond.

By **Maxwell** · Reviewed by **Maxwell** (First Contact Practitioner, HCPC PH128483, CSP Member) · 2026-06-21 · [livingwitharthritis.org.uk](https://livingwitharthritis.org.uk)

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Falls are among the most common and serious health risks facing older adults, particularly those who are frail. In the UK, falls cause more than 2.5 million injuries annually in people aged 65 and over. Beyond the immediate risk of fracture or head injury, falls can trigger a loss of confidence that leads to reduced activity, which in turn increases frailty and future fall risk — a cycle worth interrupting early. The encouraging news is that most falls are preventable with a combination of practical and physical interventions.



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## Why Fall Prevention Deserves Attention

Falls are not an inevitable part of ageing. While fall risk does increase with age, particularly after 80, the majority of falls result from modifiable risk factors — factors you can do something about. Even small reductions in fall risk, when applied consistently across the population, prevent thousands of serious injuries and fractures annually.

A hip fracture, one of the most serious consequences of a fall, often marks a turning point in an older person's life. Recovery is slow and difficult, mobility may be permanently reduced, and many people lose independence as a result. This makes prevention not just about avoiding the immediate injury, but about maintaining independence and quality of life long-term.

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## Common Risk Factors for Falls



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Fall risk increases with muscle weakness, balance problems, certain medications (particularly those affecting blood pressure or alertness), vision impairment, and home hazards such as poor lighting or loose rugs. Chronic conditions including arthritis, neuropathy, and Parkinson's disease can also affect balance and gait, raising fall risk further.

Many falls involve multiple risk factors. Someone with weak leg muscles, taking blood pressure medication that causes dizziness, living in a home with poor lighting, and wearing slippers instead of proper shoes is at much higher risk than someone with just one of these factors. This means fall prevention often requires addressing several areas simultaneously.

Vision is particularly important. Uncorrected or partially corrected vision problems are surprisingly common and modifiable. An eye test and new glasses, where needed, can meaningfully reduce fall risk. Similarly, hearing loss can affect balance, and addressing it

through hearing aids can help.

Medications deserve special attention. Blood pressure-lowering medications, sedating medications, and medications that affect balance (certain anticonvulsants, some pain medications) all increase fall risk. A doctor or pharmacist can review medication side effects and sometimes adjust dosages or switch to alternatives with fewer effects on balance or alertness.



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## Exercises to Improve Balance

Balance and strength training are among the most effective tools for reducing fall risk. Tai chi has strong evidence behind it for improving balance in older adults and reducing fall rates. Simple exercises such as standing on one leg while holding onto a sturdy support, heel-to-toe walking, and gentle resistance training for the legs can be practiced regularly, ideally under the

guidance of a physical therapist when starting out.

Strengthening the legs is particularly important since they bear your weight and control balance. Exercises like sit-to-stands (standing up from a chair and sitting back down repeatedly), step-ups on a low step, and gentle squats all build leg strength. These exercises should be done safely, with support available if balance is poor.

Balance practice itself is important. Standing on one leg, walking heel-to-toe, or following a line on the ground — these activities, done safely, help train the balance systems. Even gentle tai chi or yoga, done with appropriate modifications and support, can improve balance meaningfully.

The key is consistency. A physical therapist can design a personalised programme suited to your specific balance and strength challenges.

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## **Making the Home Safer**

Many falls happen at home, which makes home safety a high-impact area for prevention. Removing loose rugs or securing them with non-slip backing, improving lighting in hallways and staircases, installing grab bars in the bathroom, keeping frequently used items within easy reach, and clearing walkways of clutter and cords can substantially reduce hazards.

Bathrooms deserve particular attention since they're slippery and often poorly lit. Installing grab bars near the toilet and in the shower, using a shower seat, placing a non-slip mat in the bath, and improving lighting can all make a significant difference.

Bedside considerations matter too. A clear, lit path to the bathroom at night reduces fall risk. Some people benefit from a bedside commode or urinal to reduce nighttime toilet visits. A nightlight or motion-activated lighting can prevent trips.

Footwear is surprisingly important. Properly fitting shoes with good grip and support, worn consistently (not slippers), reduce fall risk. Good footwear should have a supportive sole and heel, not be too loose or too tight, and provide traction.

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## **Working With Your Healthcare Team**

A doctor or pharmacist can review medications for any that may increase fall risk and adjust where appropriate. Regular vision checks, appropriate footwear with good grip, and the use of mobility aids such as a cane or walker when recommended all support safer movement. Some healthcare providers also offer formal fall-risk assessments, which combine a physical

evaluation with a home safety review.

Fall prevention works best as a layered approach — combining stronger muscles and better balance with a safer environment and careful medication review. Taking these steps proactively, rather than after a fall has already occurred, offers the greatest benefit. If you've had a fall, or if you're worried about fall risk, discuss it with your healthcare provider. Early intervention can prevent future falls and the serious injuries they can cause.

**\*\*This article is for general information only and is not a substitute for professional medical advice. Always consult with your healthcare provider before starting new exercise programmes or making changes to your home.\*\***

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## Q&A

### **Question: What is the key thing to know about preventing falls: a guide for frail and at-risk adults?**

**Short answer:** Falls are a serious health risk for older adults, but most are preventable. Modifiable risk factors include muscle weakness, balance problems, certain medications, poor vision, and home hazards like loose rugs. Addressing these through exercise, home safety improvements such as grab bars and better lighting, vision checks, and medication reviews with a doctor can significantly reduce fall risk.

### **Question: When should I seek professional advice?**

**Short answer:** If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

### **Question: Where can I get free arthritis support in the UK?**

**Short answer:** Versus Arthritis: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at [livingwitharthritis.org.uk](https://livingwitharthritis.org.uk).

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General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis UK · [livingwitharthritis.org.uk](https://livingwitharthritis.org.uk)